

As this preparations contains Benzyl Alcohol, its use should be avoided in children under 2 years of age. Not to be used in neonates.

DRUG INTERACTIONS:

Combinations containing any of the following medications depending on the amount present, may also interact with this medication.

Acetaminophen; Alcohol or Anti-inflammatory drugs, nonsteroidal (NSAID's); Aminoglutethimide; Amphotericin B, parenteral or Carbonic anhydrase inhibitors; Anabolic steroids or Androgens; Antacids; Anticoagulants, coumarin, or indandione – derivative, or Heparin or Streptokinase or Urokinase; Antidepressants tricyclic; Antidiabetic agents, oral or Insulin; Anticholinergics, especially atropine and related compounds; Asparaginase; Contraceptives, oral, estrogen-containing, or Oestrogens; Digitalis glycosides; Diuretics; Ephedrine; Folic Acid; Hepatic enzyme inducing agents; Immunosuppressant agents, other; Isoniazide; Iopendylate or Metrizamide; Mitotane; Neuromuscular blocking agents, nondepolarizing; Potassium supplements; Ritodrine; Salicylates; Sodium – containing medication or foods; Somatrem or Somatropin; Streptozocin; Antithyroid agents or Thyroid hormones; Troleandomycin; Vaccines, live virus or other immunizations

RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION:

Usual adult dose:

By deep intramuscular injection, 40 mg to 80 mg of acetone of depot effect, repeated at intervals (usually at four-week interval) according to the patient's response.

Intra-articular injection, 2.5 to 15 mg.

Usual paediatric dose:

Children up to 6 years of age: Use is not recommended.

Children 6 to 12 years of age:

Deep intramuscular injection, 40 mg, repeated at four-week interval if necessary. 30 to 200 mcg (0.03 to 0.2 mg) per kg of body weight or 1 to 6.25 mg per square meter of body surface, repeated at one to seven-day intervals.

Intra-articular injection, 2.5 to 15 mg, repeated as needed.

It is recommended that intra-articular injection be repeated no more often than once every 3 weeks. Frequent repeated injections may cause joint damage. Administration of a local anesthetic concurrently with intra-articular injection of a corticosteroid may reduce the pain of injection and provide immediate relief of symptoms. However, a post injection flare of pain may occur when the local anesthetic effect subsides.

Flurocort injections are not for intravenous use.

Strict aseptic precautions should be observed. Since the duration of effect is variable, subsequent doses should be given when symptoms recur and not at per interval.

It should be emphasised that dosage requirements are variable

and must be individualised on the basis of the disease under treatment and the response of patient.

SYMPTOMS AND TREATMENT FOR OVERDOSE, AND ANTIDOTE(S):

The adverse effects of corticosteroids are nearly always due to their use in excess of normal physiological requirements. They should be treated symptomatically, where the dosage being reduced or the drug slowly withdrawn.

Treatment of adverse effects:

For gastro-intestinal effects - Administration of antacids between meals may relieve indigestion or mild gastrointestinal irritation that may occur during parenteral therapy. However, the efficacy of antacids or other antiulcer medications in preventing severe gastrointestinal problems. Such as ulceration, hemorrhage, and/or bowel perforation, during corticosteroids therapy has not been established.

Mental depression or psychoses - If possible, decrease corticosteroids dosage or discontinue therapy. Phenothiazines may be administered if necessary. Lithium has been recommended. Some patients may require electroconvulsive therapy if severe depression persists. Tricyclic anti-depressants do not relieve and may exacerbate, corticosteroids-induced mental disturbances and not to be used.

For withdrawal effects (non-HPA axis suppression) - Administration of aspirin or another NSAIDs may alleviate some of the symptoms of this condition.

STORAGE CONDITIONS, USER INSTRUCTIONS AND PHARMACEUTICAL PRECAUTIONS:

Store below 30°C. Protect from light and freezing. Shake well before used.

PACKING / PACK SIZES:

For 40 mg/ml: 40 mg/ ml in 5 ml vial, 10 and 25 vials in a carton

SHELF LIFE: 3 years

MANUFACTURER AND PRODUCT REGISTRATION HOLDER:

SM PHARMACEUTICALS SDN BHD (218620-M)

Lot 88, Sungai Petani Industrial Estate

08000 Sungai Petani, Kedah Darul Aman, Malaysia.

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FLUROCORT (TRIAMCINOLONE ACETONIDE) INJECTION

DESCRIPTION:

Almost white, viscous, oily suspension.

COMPOSITION:

Each ml of suspension of injection contains:

For 40 mg/ml: Triamcinolone Acetonide 40 mg / ml

ACTIONS AND MODE OR MECHANISMS OF ACTION:

Corticosteroids - Diffuse across cell membranes and complex with specific cytoplasmic receptors. These complexes then enter the cell nucleus, bind to DNA (chromatin), and stimulate transcription of mRNA (messenger RNA) and subsequent protein synthesis of various enzymes thought to be ultimately responsible for two categories of effects of systemic Corticosteroids.

Anti-inflammatory - Triamcinolone decreases or prevents tissue responses to inflammatory processes, thereby reducing development of symptoms of inflammation without affecting the underlying cause. Triamcinolone inhibits accumulation of inflammatory cells including macrophages and leukocytes, at sites of inflammation. It inhibits phagocytosis, lysosomal enzyme release, and synthesis and/or release of several chemical mediators of inflammation.

Immunosuppressant - May involve prevention or suppression of cell-mediated (delayed hypersensitivity) immune reactions as well as more specific actions affecting the immune response. Triamcinolone reduces the concentration of thymus-dependent lymphocytes (T-lymphocytes), monocytes and eosinophils. It is also decrease binding of immunoglobulin to cell surface receptors and inhibit the synthesis and/or release of or interleukins, thereby decreasing T-lymphocyte blastogenesis and reducing expansion of the primary immune response.

PHARMACOLOGY (SUMMARY OF PHARMACODYNAMICS AND PHARMACOKINETICS):

Absorption - IM: Slowly but completely absorbed

Biotransformation - primarily hepatic (rapid); also renal and tissue; mostly to inactive metabolites.

Duration of action - depends upon the solubility of the dosage form as well as the biological (tissue) half-life.

Elimination - Renal, mainly as inactive metabolites

INDICATIONS:

Where oral therapy is not feasible or is temporarily undesirably in the judgement of the physician, Flurocort is indicated for Intramuscular use as follows:

Endocrine disorders: Nonsuppurative thyroiditis.

Rheumatic disorders: As adjunctive therapy for short term administration (to tide the patient over an acute episode of exacerbation) in post-traumatic osteoarthritis; synovitis of osteoarthritis; rheumatoid arthritis; acute and subacute bursitis;

epicondylitis; acute non-specific tenosynovitis; acute gouty arthritis, psoriatic arthritis; ankylosing spondylitis; juvenile rheumatoid arthritis.

Collagen Diseases: During an exacerbation as maintenance therapy in selected cases of systemic lupus erythematosus: Acute rheumatic carditis.

Dermatologic Disease: Pemphigus, severe erythema multiforme (Steven-Johnson Syndrome), exfoliative dermatitis, bullous dermatitis herpetiformis, severe seborrheic dermatitis; severe psoriasis.

Allergic States: Control of severe or incapacitating allergic condition intractable to adequate trials of conventional treatment in bronchial asthma; contact dermatitis; atopic dermatitis, seasonal or perennial allergic rhinitis.

Ophthalmic Diseases: Severe chronic allergic and inflammatory processes involving the eye, e.g. herpes zoster ophthalmicus; iritis, indocyclitis, chorioretinitis; diffuse posterior uveitis and choroditis; optic neuritis; sympathetic ophthalmia; anterior segment inflammation.

Gastrointestinal disease: To tide the patient over a critical period of disease in ulcerative colitis (systemic therapy), regional enteritis (systemic therapy).

Respiratory Diseases: Symptomatic sarcoidosis; berylliosis; aspiration pneumonitis.

Hemolytic Disorders: Acquired (autoimmune) haemolytic anaemia.

Neoplastic Diseases: For palliative management of leukaemia and lymphomas in adults and acute leukaemia of childhood.

Edematous State: To include diuresis or remission of proteinuria in the nephrotic syndrome, without uremia, of the idiopathic type or that due to lupus erythematosus.

Intra - Articular: Flucort IM is indicated for intra-articular administration and for injections into tendon sheaths, as adjunctive therapy for short- term administration (to tide the patient over an acute episodes or exacerbation) in synovitis of osteoarthritis, rheumatoid arthritis, acute and sub acute bursitis; acute gouty arthritis; epicondylitis, acute non specific tenosynovitis, post-traumatic osteoarthritis.

CONTRAINDICATIONS:

Unless considered life saving, systemic administration of corticosteroids is contra-indicated in-patients with peptic ulcer, osteoporosis, psychoses, or severe psychoneuroses. Corticosteroids are contraindicated in-patients with systemic fungal infections. Intramuscular corticosteroid preparations are contraindicated for idiopathic thrombocytopenic purpura (ITP). It is also contraindicated in patients with hypersensitivity to active or inactive ingredients.

SIDE EFFECTS/ADVERSE REACTIONS:

Overdosage or prolonged use may exaggerate some of the normal physiological actions; hypertension, sodium and water retention, potassium lost, and muscle weakness. Diabetes and osteoporosis which is a danger, particularly in the elderly, as it may result in vertebral collapse. Mental disturbances, euphoria

and peptic ulceration. In children, administration of corticosteroids may result in suppression of growth.

Triamcinolone has a particular tendency to cause a proximal myopathy and should be avoided in chronic therapy.

Dermatologic: Impaired wound healing, thin fragile skin petechiae and ecchymoses, facial erythema, increased sweating, purpura striae, hirsutism, acneiform eruptions, lupus erythematosus-like lesions and suppressed reactions to skin tests.

Neurological: Convulsions, increased intracranial pressure with papilledema (Pseudotumor cerebri) usually after treatment, vertigo, headache, neuritis or paresthesias and aggravation of pre existing psychiatric conditions.

Endocrine: Menstrual irregularities, development of the cushingoid state; suppression of growth in children; secondary adrenocortical and pituitary unresponsiveness, particular in time of stress (e.g. trauma, surgery or illness); decreased carbohydrate tolerance; manifestation of latent diabetes mellitus and increased requirements for insulin or oral hypoglycaemic agents in diabetic.

Ophthalmic: Posterior subcapsular cataracts, increased intraocular pressure, glaucoma and exophthalmos.

Metabolic: Hyperglycaemia, glycosuria and negative nitrogen due to protein catabolism.

Following intramuscular administration: Severe pain has been reported in few cases. Sterile abscess formation, subcutaneous and cutaneous atrophy, hyperpigmentation, hypopigmentation and charcot-like atrophy have also occurred.

Anaphylactic reaction: Pharmacologic doses of glucocorticoids lower resistance to infection; the patient may be predisposed to systemic infections during, and for the time following, therapy. Increased susceptibility to infection may occur with short-term high-dose use ("pulse" therapy) as well as with more prolonged use. Also, symptoms of onset or progression of infections may be masked.

Intra-articular injection: Undesirable reactions have included post-injection flare, transient pain, occasional irritation at the injection site, sterile abscess formation, hyper- and hypopigmentation, charcot-like arthropathy and occasional brief increase in joint discomfort.

It should be noted that there have been several reports of joint damage after the intra-articular injection of corticosteroids into load-bearing joints.

PRECAUTIONS/WARNINGS:

It should be used only with great caution in the presence of congestive heart failure or hypertension, in-patients with diabetes mellitus, infectious diseases, chronic renal failure, and uraemia, and elderly persons. Patients with active or doubtfully quiescent tuberculosis should not be given corticosteroid except, very rarely.

The lowest dose of corticosteroids should not be used to control the condition being treated. A gradual reduction in dosage should be made when possible.

Menstrual irregularities may occur, and this possibility should be mentioned to female patients past menarche.

Patients who are receiving corticosteroid therapy are subjected to unusual stress, increased dosage of rapidly acting corticosteroids is indicated before, during and after the stressful situation.

Patients should not be vaccinated against chickenpox while on corticosteroid therapy.

Other immunisation procedure should not be undertaken in-patients who are on corticosteroids, especially on high dose, because of possible hazard of neurological complications and a lack of antibody response.

There is enhanced corticosteroid effect in-patients with hypothyroidism and in those with cirrhosis.

Pregnancy: Corticosteroids are not recommended for pregnant patients, particularly in the first trimester or for nursing mothers, except when the disease for which they are indicated warrants their use. There is inadequate safety evidence in human pregnancy.

Elderly: Treatment of elderly patients, particularly in long term, should be planned bearing in mind the more serious consequences of the common side effects of corticosteroids in old age especially osteoporosis, diabetes, hypertension, susceptibility to infection and thinning of the skin.

Children: Flucort is not recommended for children under 6 years. Growth and development of children on prolonged corticosteroid therapy should be carefully observed.

Myasthenia gravis: Muscle weakness may be increased initially, possibly leading to respiratory distress; patients should be hospitalized, and respiratory support should be immediately available, when glucocorticoid therapy is initiated.

Psychiatric disturbances: Psychic derangements may appear when corticosteroids are used. These may range from euphoria, insomnia, mood swings, personality changes and severe depression, to frank psychotic manifestation. Existing emotional instability or psychotic tendencies may also be aggravated by corticosteroids.

Adequate protein intake: During prolonged therapy, a liberal protein intake is essential for counteracting the tendency to gradual weight loss sometimes associated with negative nitrogen balance, wasting and weakness of skeletal muscle.

Increased calcium excretion: All corticosteroids increase calcium excretion. Avoid abrupt cessation of corticosteroids.

Intra-articular injection: This medication should not be used prior to arthroplasty of joints (increased risk of infection); blood clotting disorders (risk of intra- and extra-articular hemorrhage); intra-articular fracture (healing may be retarded); current or history of periarticular infection (may be exacerbated or reactivated); non-arthritic juxta-articular osteoporosis (may be exacerbated) and unstable joints.

Warnings:

Following intra-articular injection, the injected joints should not be overused even if pain is relieved, because of the increased risk of joint damage or deterioration. It is recommended that weight-bearing joints be rested for 24 to 48 hours post-injection.